



Tests you can trust

Name : Staicy (46Y/F)

Date : 15 Jun 2025

Test Asked : Aarogyam C Pro With Utsh

Report Status: Complete Report



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Controlled
Sample Logistics



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Daily



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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]

NAME : STAICY (46Y/F)
REF. BY : SELF
TEST ASKED : AAROGYAM C PRO WITH UTSH

SAMPLE COLLECTED AT :
(5770026562),SHIVA HITECH DIAGNOSTIC
CENTER,177/C 3RD MAIN ROAD P J EXTENSION
DAVANAGERE KARNATAKA,577002

Report Availability Summary

Note: Please refer to the table below for status of your tests.

✓ **12** Ready ⚠ **0** Ready with Cancellation 🔄 **0** Processing ✗ **0** Cancelled in Lab

TEST DETAILS

REPORT STATUS

AAROGYAM C PRO WITH UTSH

Ready ✓

CHLORIDE

Ready ✓

HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)

Ready ✓

SODIUM

Ready ✓

TESTOSTERONE

Ready ✓

HBA PROFILE

Ready ✓

HEMOGRAM - 6 PART (DIFF)

Ready ✓

LIVER FUNCTION TESTS

Ready ✓

IRON DEFICIENCY PROFILE

Ready ✓

KIDPRO

Ready ✓

LIPID PROFILE

Ready ✓

T3-T4-USTSH

Ready ✓

VITAMIN D TOTAL AND B12 COMBO

Ready ✓

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Summary Report**Tests outside reference range**

TEST NAME	OBSERVED VALUE	UNITS	Bio. Ref. Interval.
COMPLETE HEMOGRAM			
LYMPHOCYTE	19.7	%	20-40
MEAN CORP.HEMO.CONC(MCHC)	29.4	g/dL	31.5-34.5
MEAN CORPUSCULAR HEMOGLOBIN(MCH)	26.2	pq	27.0-32.0
MEAN PLATELET VOLUME(MPV)	12.1	fL	6.5-12
PLATELET DISTRIBUTION WIDTH(PDW)	15.7	fL	9.6-15.2
RED CELL DISTRIBUTION WIDTH (RDW-CV)	14.9	%	11.6-14.0
RED CELL DISTRIBUTION WIDTH - SD(RDW-SD)	48.2	fL	39.0-46.0
TOTAL RBC	4.88	X 10 ⁶ /μL	3.8-4.8
IRON DEFICIENCY			
% TRANSFERRIN SATURATION	10.55	%	13 - 45
IRON	41.96	μg/dL	50 - 170
LIPID			
TC/ HDL CHOLESTEROL RATIO	2.5	Ratio	3 - 5
LIVER			
SERUM GLOBULIN	3.66	gm/dL	2.5-3.4
VITAMIN			
25-OH VITAMIN D (TOTAL)	20.5	ng/mL	30-100

Disclaimer: The above listed is the summary of the parameters with values outside the BRI. For detailed report values, parameter correlation and clinical interpretation, kindly refer to the same in subsequent pages.

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First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]**NAME** : STAICY (46Y/F)**REF. BY** : SELF**TEST ASKED** : HBA PROFILE,HEMOGRAM**SAMPLE COLLECTED AT :**(5770026562),SHIVA HITECH DIAGNOSTIC
CENTER,177/C 3RD MAIN ROAD P J EXTENSION
DAVANAGERE KARNATAKA,577002

TEST NAME	TECHNOLOGY	VALUE	UNITS
HbA1c	H.P.L.C	5.5	%

Bio. Ref. Interval. :**As per ADA Guidelines**Below 5.7% : Normal
5.7% - 6.4% : Prediabetic
>=6.5% : Diabetic**Guidance For Known Diabetics**Below 6.5% : Good Control
6.5% - 7% : Fair Control
7.0% - 8% : Unsatisfactory Control
>8% : Poor Control**Method :** Fully Automated H.P.L.C method

AVERAGE BLOOD GLUCOSE (ABG) CALCULATED 111 mg/dL

Bio. Ref. Interval. :90 - 120 mg/dl : Good Control
121 - 150 mg/dl : Fair Control
151 - 180 mg/dl : Unsatisfactory Control
> 180 mg/dl : Poor Control**Method :** Derived from HBA1c values**Please correlate with clinical conditions.****Sample Collected on (SCT)** : 14 Jun 2025 19:25**Sample Received on (SRT)** : 15 Jun 2025 08:54**Report Released on (RRT)** : 15 Jun 2025 10:24**Sample Type** : EDTA Whole Blood**Labcode** : 1506062945/HBDN1**Barcode** : DK363888

Dr Syeda Sumaiya MD(Path)

Scan QR code to verify authenticity of reported results; active for 30 days from release time.

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TEST ASKED : HBA PROFILE,HEMOGRAM

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TEST NAME	METHODOLOGY	VALUE	UNITS	Bio. Ref. Interval.
HEMOGLOBIN	SLS-Hemoglobin Method	12.8	g/dL	12.0-15.0
Hematocrit (PCV)	CPH Detection	43.5	%	36.0-46.0
Total RBC	HF & EI	4.88	X 10⁶/μL	3.8-4.8
Mean Corpuscular Volume (MCV)	Calculated	89.1	fL	83.0-101.0
Mean Corpuscular Hemoglobin (MCH)	Calculated	26.2	pq	27.0-32.0
Mean Corp.Hemo. Conc (MCHC)	Calculated	29.4	g/dL	31.5-34.5
Red Cell Distribution Width - SD (RDW-SD)	Calculated	48.2	fL	39.0-46.0
Red Cell Distribution Width (RDW - CV)	Calculated	14.9	%	11.6-14.0
RED CELL DISTRIBUTION WIDTH INDEX (RDWI)	Calculated	272	-	*Refer Note below
MENTZER INDEX	Calculated	18.3	-	*Refer Note below
TOTAL LEUCOCYTE COUNT (WBC)	HF & FC	7.47	X 10 ³ / μL	4.0 - 10.0
DIFFERENTIAL LEUCOCYTE COUNT				
Neutrophils Percentage	Flow Cytometry	75.6	%	40-80
Lymphocytes Percentage	Flow Cytometry	19.7	%	20-40
Monocytes Percentage	Flow Cytometry	2.8	%	2-10
Eosinophils Percentage	Flow Cytometry	1.1	%	1-6
Basophils Percentage	Flow Cytometry	0.5	%	0-2
Immature Granulocyte Percentage (IG%)	Flow Cytometry	0.3	%	0.0-0.4
Nucleated Red Blood Cells %	Flow Cytometry	0.01	%	0.0-5.0
ABSOLUTE LEUCOCYTE COUNT				
Neutrophils - Absolute Count	Calculated	5.65	X 10 ³ / μL	2.0-7.0
Lymphocytes - Absolute Count	Calculated	1.47	X 10 ³ / μL	1.0-3.0
Monocytes - Absolute Count	Calculated	0.21	X 10 ³ / μL	0.2 - 1.0
Basophils - Absolute Count	Calculated	0.04	X 10 ³ / μL	0.02 - 0.1
Eosinophils - Absolute Count	Calculated	0.08	X 10 ³ / μL	0.02 - 0.5
Immature Granulocytes (IG)	Calculated	0.02	X 10 ³ / μL	0.0-0.3
Nucleated Red Blood Cells	Calculated	0.01	X 10 ³ / μL	0.0-0.5
PLATELET COUNT	HF & EI	249	X 10 ³ / μL	150-410
Mean Platelet Volume (MPV)	Calculated	12.1	fL	6.5-12
Platelet Distribution Width (PDW)	Calculated	15.7	fL	9.6-15.2
Platelet to Large Cell Ratio (PLCR)	Calculated	40.9	%	19.7-42.4
Plateletcrit (PCT)	Calculated	0.3	%	0.19-0.39

Remarks : Alert!!! Predominantly normocytic normochromic with ovalocytes. Platelets:Appear adequate in smear.

***Note - Mentzer index (MI), RDW-CV and RDWI are hematological indices to differentiate between Iron Deficiency Anemia (IDA) and Beta Thalassemia Trait (BTT). MI >13, RDWI >220 and RDW-CV >14 more likely to be IDA. MI <13, RDWI <220, and RDW-CV <14 more likely to be BTT. Suggested Clinical correlation. BTT to be confirmed with HB electrophoresis if clinically indicated.**

Method : Fully automated bidirectional analyser (6 Part Differential SYSMEX XN-1000)

(Reference : *FC- flowcytometry, *HF- hydrodynamic focussing, *EI- Electric Impedence, *Hb- hemoglobin, *CPH- Cumulative pulse height)

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DAVANAGERE KARNATAKA, 577002

TEST NAME	TECHNOLOGY	VALUE	UNITS
25-OH VITAMIN D (TOTAL)	E.C.L.I.A	20.5	ng/mL

Bio. Ref. Interval. :

Deficiency : ≤ 20 ng/ml || Insufficiency : 21-29 ng/ml

Sufficiency : ≥ 30 ng/ml || Toxicity : > 100 ng/ml

Clinical Significance:

Vitamin D is a fat soluble vitamin that has been known to help the body absorb and retain calcium and phosphorous; both are critical for building bone health.

Decrease in vitamin D total levels indicate inadequate exposure of sunlight, dietary deficiency, nephrotic syndrome.

Increase in vitamin D total levels indicate Vitamin D intoxication.

Specifications: Precision: Intra assay (%CV):9.20%, Inter assay (%CV):8.50%

Kit Validation Reference : Holick M. Vitamin D the underappreciated D-Lightful hormone that is important for Skeletal and cellular health Curr Opin Endocrinol Diabetes 2002;9(1):87-98.

Method : Fully Automated Electrochemiluminescence Competitive Immunoassay

VITAMIN B-12	E.C.L.I.A	337	pg/mL
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Bio. Ref. Interval. :

Normal: 197-771 pg/ml

Clinical significance :

Vitamin B12 or cyanocobalamin, is a complex corrinoid compound found exclusively from animal dietary sources, such as meat, eggs and milk. It is critical in normal DNA synthesis, which in turn affects erythrocyte maturation and in the formation of myelin sheath. Vitamin-B12 is used to find out neurological abnormalities and impaired DNA synthesis associated with macrocytic anemias. For diagnostic purpose, results should always be assessed in conjunction with the patients medical history, clinical examination and other findings.

Specifications: Intra assay (%CV):2.6%, Inter assay (%CV):2.3 %

Kit Validation Reference : Thomas L. Clinical Laboratory Diagnostics : Use and Assessment of Clinical Laboratory Results 1st Edition, TH Books-Verl-Ges, 1998:424-431

Method : Fully Automated Electrochemiluminescence Competitive Immunoassay

Please correlate with clinical conditions.

Sample Collected on (SCT) : 14 Jun 2025 19:25

Sample Received on (SRT) : 15 Jun 2025 08:57

Report Released on (RRT) : 15 Jun 2025 12:07

Sample Type : SERUM

Labcode : 1506063062/HBDN1 Dr Syeda Sumaiya MD(Path)

Barcode : DU040865



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TEST NAME	TECHNOLOGY	VALUE	UNITS
TESTOSTERONE	E.C.L.I.A	7.78	ng/dL
Bio. Ref. Interval. :-			

6 - 82

Clinical Significance: Clinical evaluation of serum testosterone, along with serum LH, assists in evaluation of Hypogonadal males. Major causes of lowered testosterone in males include Hypogonadotropic hypogonadism, testicular failure Hyperprolactinemia, Hypopituitarism some types of liver and kidney diseases and critical illness.

Specifications: Precision: Intra assay (%CV): 11.50 %, Inter assay (%CV): 5.70%; Sensitivity: 7 ng/dL.

Kit Validation Reference: Wilson JD Foster DW (Eds) Williams Textbook of Endocrinology 8th Edition WB Saunders Philadelphia Pennsylvania.

Note : The Biological Reference Range mentioned is specific to the age group and gender. Kindly correlate clinically.

Please correlate with clinical conditions.

Method:- Fully Automated Electrochemiluminescence Competitive Immunoassay

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TEST NAME	TECHNOLOGY	VALUE	UNITS
HIGH SENSITIVITY C-REACTIVE PROTEIN (HS-CRP)	IMMUNOTURBIDIMETRY	1.8	mg/L
Bio. Ref. Interval. :-			

< 1.00 - Low Risk

1.00 - 3.00 - Average Risk

>3.00 - 10.00 - High Risk

> 10.00 - Possibly due to Non-Cardiac Inflammation

Disclaimer: Persistent unexplained elevation of HSCRP >10 should be evaluated for non-cardiovascular etiologies such as infection, active arthritis or concurrent illness.

Clinical significance:

High sensitivity C- reactive Protein (HSCRP) can be used as an independent risk marker for the identification of Individuals at risk for future cardiovascular Disease. A coronary artery disease risk assessment should be based on the average of two hs-CRP tests, ideally taken two weeks apart.

Kit Validation Reference:

1. Clinical management of laboratory data in medical practice 2003-2004, 207(2003).

2. Tietz : Textbook of Clinical Chemistry and Molecular diagnostics : Second edition : Chapter 47: Page no. 1507- 1508.

Please correlate with clinical conditions.**Method:-** FULLY AUTOMATED LATEX AGGLUTINATION – BECKMAN COULTER**Sample Collected on (SCT)** : 14 Jun 2025 19:25**Sample Received on (SRT)** : 15 Jun 2025 08:57**Report Released on (RRT)** : 15 Jun 2025 12:07**Sample Type** : SERUM**Labcode** : 1506063062/HBDN1 Dr Syeda Sumaiya MD(Path)**Barcode** : DU040865

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TEST NAME	TECHNOLOGY	VALUE	UNITS
IRON Bio. Ref. Interval. : Male : 65 - 175 Female : 50 - 170 Method : Ferrozine method without deproteinization	PHOTOMETRY	41.96	µg/dL
TOTAL IRON BINDING CAPACITY (TIBC) Bio. Ref. Interval. : Male: 225 - 535 µg/dl Female: 215 - 535 µg/dl Method : Spectrophotometric Assay	PHOTOMETRY	397.73	µg/dL
% TRANSFERRIN SATURATION Bio. Ref. Interval. : 13 - 45 Method : Derived from IRON and TIBC values	CALCULATED	10.55	%
UNSAT. IRON-BINDING CAPACITY (UIBC) Bio. Ref. Interval. : 162 - 368 Method : SPECTROPHOTOMETRIC ASSAY	PHOTOMETRY	355.77	µg/dL

Please correlate with clinical conditions.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL CHOLESTEROL	PHOTOMETRY	135	mg/dL	< 200
HDL CHOLESTEROL - DIRECT	PHOTOMETRY	53	mg/dL	40-60
HDL / LDL RATIO	CALCULATED	0.66	Ratio	> 0.40
LDL CHOLESTEROL - DIRECT	PHOTOMETRY	81	mg/dL	< 100
TRIG / HDL RATIO	CALCULATED	1.19	Ratio	< 3.12
TRIGLYCERIDES	PHOTOMETRY	63	mg/dL	< 150
TC/ HDL CHOLESTEROL RATIO	CALCULATED	2.5	Ratio	3 - 5
LDL / HDL RATIO	CALCULATED	1.5	Ratio	1.5-3.5
NON-HDL CHOLESTEROL	CALCULATED	82	mg/dL	< 160
VLDL CHOLESTEROL	CALCULATED	12.58	mg/dL	5 - 40

Please correlate with clinical conditions.

Method :

CHOL - Cholesterol Oxidase, Esterase, Peroxidase
HCHO - Direct Enzymatic Colorimetric
HD/LD - Derived from HDL and LDL values.
LDL - Direct Measure
TRI/H - Derived from TRIG and HDL Values
TRIG - Enzymatic, End Point
TC/H - Derived from serum Cholesterol and Hdl values
LDL/ - Derived from serum HDL and LDL Values
NHDH - Derived from serum Cholesterol and HDL values
VLDL - Derived from serum Triglyceride values

***REFERENCE RANGES AS PER NCEP ATP III GUIDELINES:**

TOTAL CHOLESTEROL	(mg/dl)	HDL	(mg/dl)	LDL	(mg/dl)	TRIGLYCERIDES	(mg/dl)
DESIRABLE	<200	LOW	<40	OPTIMAL	<100	NORMAL	<150
BORDERLINE HIGH	200-239	HIGH	>60	NEAR OPTIMAL	100-129	BORDERLINE HIGH	150-199
HIGH	>240			BORDERLINE HIGH	130-159	HIGH	200-499
				HIGH	160-189	VERY HIGH	>500
				VERY HIGH	>190		

Alert !!! 10-12 hours fasting is mandatory for lipid parameters. If not, values might fluctuate.

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
ALKALINE PHOSPHATASE	PHOTOMETRY	65.65	U/L	45-129
BILIRUBIN - TOTAL	PHOTOMETRY	0.4	mg/dL	0.3-1.2
BILIRUBIN -DIRECT	PHOTOMETRY	0.09	mg/dL	< 0.3
BILIRUBIN (INDIRECT)	CALCULATED	0.31	mg/dL	0-0.9
GAMMA GLUTAMYL TRANSFERASE (GGT)	PHOTOMETRY	10.3	U/L	< 38
SGOT / SGPT RATIO	CALCULATED	1.59	Ratio	< 2
ASPARTATE AMINOTRANSFERASE (SGOT)	PHOTOMETRY	18	U/L	< 31
ALANINE TRANSAMINASE (SGPT)	PHOTOMETRY	11.3	U/L	< 34
PROTEIN - TOTAL	PHOTOMETRY	7.72	gm/dL	5.7-8.2
ALBUMIN - SERUM	PHOTOMETRY	4.06	gm/dL	3.2-4.8
SERUM GLOBULIN	CALCULATED	3.66	gm/dL	2.5-3.4
SERUM ALB/GLOBULIN RATIO	CALCULATED	1.11	Ratio	0.9 - 2

Please correlate with clinical conditions.

Method :

ALKP - Modified IFCC method
BILT - Vanadate Oxidation
BILD - Vanadate Oxidation
BILI - Derived from serum Total and Direct Bilirubin values
GGT - Modified IFCC method
OT/PT - Derived from SGOT and SGPT values.
SGOT - IFCC* Without Pyridoxal Phosphate Activation
SGPT - IFCC* Without Pyridoxal Phosphate Activation
PROT - Biuret Method
SALB - Albumin Bcg¹method (Colorimetric Assay Endpoint)
SEGB - DERIVED FROM SERUM ALBUMIN AND PROTEIN VALUES
A/GR - Derived from serum Albumin and Protein values

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TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
BLOOD UREA NITROGEN (BUN)	PHOTOMETRY	15.6	mg/dL	7.94 - 20.07
UREA (CALCULATED)	CALCULATED	33.38	mg/dL	Adult : 17-43
CREATININE - SERUM	PHOTOMETRY	0.72	mg/dL	0.55-1.02
UREA / SR.CREATININE RATIO	CALCULATED	46.37	Ratio	< 52
BUN / SR.CREATININE RATIO	CALCULATED	21.67	Ratio	9:1-23:1
CALCIUM	PHOTOMETRY	9.4	mg/dL	8.8-10.6
URIC ACID	PHOTOMETRY	5.37	mg/dL	3.2 - 6.1
SODIUM	I.S.E - INDIRECT	138.84	mmol/L	136 - 145
CHLORIDE	I.S.E - INDIRECT	103.24	mmol/L	98 - 107

Please correlate with clinical conditions.

Method :

BUN - Kinetic UV Assay.
UREAC - Derived from BUN Value.
SCRE - Creatinine Enzymatic Method
UR/CR - Derived from UREA and Sr.Creatinine values.
B/CR - Derived from serum Bun and Creatinine values
CALC - Arsenazo III Method, End Point.
URIC - Uricase / Peroxidase Method
SOD - ION SELECTIVE ELECTRODE - INDIRECT
CHL - ION SELECTIVE ELECTRODE - INDIRECT

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SAMPLE COLLECTED AT :
(5770026562), SHIVA HITECH DIAGNOSTIC
CENTER, 177/C 3RD MAIN ROAD P J EXTENSION
DAVANAGERE KARNATAKA, 577002

TEST NAME	TECHNOLOGY	VALUE	UNITS	Bio. Ref. Interval.
TOTAL TRIIODOTHYRONINE (T3)	E.C.L.I.A	113	ng/dL	80-200
TOTAL THYROXINE (T4)	E.C.L.I.A	8.71	µg/dL	4.8-12.7
TSH - ULTRASENSITIVE	E.C.L.I.A	3.24	µIU/mL	0.54-5.30

Comments : ***

The Biological Reference Ranges is specific to the age group. Kindly correlate clinically.

Method :

T3, T4 - Fully Automated Electrochemiluminescence Competitive Immunoassay

USTSH - Fully Automated Electrochemiluminescence Sandwich Immunoassay

Pregnancy reference ranges for TSH/USTSH :

Trimester || T3 (ng/dl) || T4 (µg/dl) || TSH/USTSH (µIU/ml)

1st || 83.9-196.6 || 4.4-11.5 || 0.1-2.5

2nd || 86.1-217.4 || 4.9-12.2 || 0.2-3.0

3rd || 79.9-186 || 5.1-13.2 || 0.3-3.5

References :

1. Carol Devilia, C I Parhon. First Trimester Pregnancy ranges for Serum TSH and Thyroid Tumor reclassified as Benign. Acta Endocrinol. 2016; 12(2) : 242 - 243

2. Kulhari K, Negi R, Kalra DK et al. Establishing Trimester specific Reference ranges for thyroid hormones in Indian women with normal pregnancy : New light through old window. Indian Journal of Contemporary medical research. 2019; 6(4)

Disclaimer : Results should always be interpreted using the reference range provided by the laboratory that performed the test. Different laboratories do tests using different technologies, methods and using different reagents which may cause difference. In reference ranges and hence it is recommended to interpret result with assay specific reference ranges provided in the reports. To diagnose and monitor therapy doses, it is recommended to get tested every time at the same Laboratory.

Sample Collected on (SCT) : 14 Jun 2025 19:25
Sample Received on (SRT) : 15 Jun 2025 08:57
Report Released on (RRT) : 15 Jun 2025 12:07
Sample Type : SERUM
Labcode : 1506063062/HBDN1 Dr Syeda Sumaiya MD(Path)
Barcode : DU040865

PROCESSED AT :

Thyrocare,
5CA-711, 3rd Floor,
HRBR 2nd Block,
Hennur, Bengaluru-560043

Thyrocare Technologies Limited, D-37/3, TTC MIDC, Turbhe, Navi Mumbai - 400 703 9870666333 wellness@thyrocare.com

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NAME : STAICY (46Y/F)

REF. BY : SELF

TEST ASKED : AAROGYAM C PRO WITH UTSH

SAMPLE COLLECTED AT :

(5770026562), SHIVA HITECH DIAGNOSTIC
CENTER, 177/C 3RD MAIN ROAD P J EXTENSION
DAVANAGERE KARNATAKA, 577002

TEST NAME	TECHNOLOGY	VALUE	UNITS
EST. GLOMERULAR FILTRATION RATE (eGFR)	CALCULATED	104	mL/min/1.73 m ²

Bio. Ref. Interval. :-

> = 90 : Normal
60 - 89 : Mild Decrease
45 - 59 : Mild to Moderate Decrease
30 - 44 : Moderate to Severe Decrease
15 - 29 : Severe Decrease

Clinical Significance

The normal serum creatinine reference interval does not necessarily reflect a normal GFR for a patient. Because mild and moderate kidney injury is poorly inferred from serum creatinine alone. Thus, it is recommended for clinical laboratories to routinely estimate glomerular filtration rate (eGFR), a "gold standard" measurement for assessment of renal function, and report the value when serum creatinine is measured for patients 18 and older, when appropriate and feasible. It cannot be measured easily in clinical practice, instead, GFR is estimated from equations using serum creatinine, age, race and sex. This provides easy to interpret information for the doctor and patient on the degree of renal impairment since it approximately equates to the percentage of kidney function remaining. Application of CKD-EPI equation together with the other diagnostic tools in renal medicine will further improve the detection and management of patients with CKD.

Reference

Levey AS, Stevens LA, Schmid CH, Zhang YL, Castro AF, 3rd, Feldman HI, et al. A new equation to estimate glomerular filtration rate. Ann Intern Med. 2009;150(9):604-12.

Please correlate with clinical conditions.

Method:- 2021 CKD EPI Creatinine Equation

~~ End of report ~~

Sample Collected on (SCT) : 14 Jun 2025 19:25

Sample Received on (SRT) : 15 Jun 2025 08:57

Report Released on (RRT) : 15 Jun 2025 12:07

Sample Type : SERUM

Labcode : 1506063062/HBDN1 Dr Syeda Sumaiya MD(Path)

Barcode : DU040865



CONDITIONS OF REPORTING

- ✓ The reported results are for information and interpretation of the referring doctor only.
- ✓ It is presumed that the tests performed on the specimen belong to the patient; named or identified.
- ✓ Results of tests may vary from laboratory to laboratory and also in some parameters from time to time for the same patient.
- ✓ Should the results indicate an unexpected abnormality, the same should be reconfirmed.
- ✓ Only such medical professionals who understand reporting units, reference ranges and limitations of technologies should interpret results.
- ✓ This report is not valid for medico-legal purpose.
- ✓ Neither Thyrocare, nor its employees/representatives assume: (a) any liability, responsibility for any loss or damage that may be incurred by any person as a result of presuming the meaning or contents of the report, (b) any claims of any nature whatsoever arising from or relating to the performance of the requested tests as well as any claim for indirect, incidental or consequential damages. The total liability, in any case, of Thyrocare shall not exceed the total amount of invoice for the services provided and paid for.
- ✓ Thyrocare Discovery video link :- <https://youtu.be/nbdYeRgYyQc>

EXPLANATIONS

- ✓ Majority of the specimen processed in the laboratory are collected by Pathologists and Hospitals we call them as "Clients".
- ✓ **Name** - The name is as declared by the client and recored by the personnel who collected the specimen.
- ✓ **Ref.Dr** - The name of the doctor who has recommended testing as declared by the client.
- ✓ **Labcode** - This is the accession number in our laboratory and it helps us in archiving and retrieving the data.
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- ✓ **RRT** - Report Releasing Time - The time when our pathologist has released the values for Reporting.
- ✓ **Reference Range** - Means the range of values in which 95% of the normal population would fall.

SUGGESTIONS

- ✓ Values out of reference range requires reconfirmation before starting any medical treatment.
- ✓ Retesting is needed if you suspect any quality shortcomings.
- ✓ Testing or retesting should be done in accredited laboratories.
- ✓ For suggestions, complaints, clinical support or feedback, write to us at **customersupport@thyrocare.com** or call us on **022-3090 0000**

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Tests you can trust

Name : Staice (46Y/F)

Date : 19 Jun 2025

Test Asked : Accp

Report Status: Complete Report



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NAME
REF. BY
TEST ASKED

: STAICE (46Y/F)
: SELF
: ACCP

SAMPLE COLLECTED AT :
(5770026562),SHIVA HITECH DIAGNOSTIC
CENTER,177/C 3RD MAIN ROAD P J EXTENSION
DAVANAGERE KARNATAKA,577002

Report Availability Summary

Note: Please refer to the table below for status of your tests.

✔ 1 Ready

🟡 0 Ready with Cancellation

🔄 0 Processing

✖ 0 Cancelled in Lab

TEST DETAILS	REPORT STATUS
ANTI CCP (ACCP)	Ready ✔

First National Diagnostic Chain to have 100% of its Labs with NABL Accreditation[#]

NAME : STAICE (46Y/F)

REF. BY : SELF

TEST ASKED : ACCP

SAMPLE COLLECTED AT :

(5770026562),SHIVA HITECH DIAGNOSTIC
CENTER,177/C 3RD MAIN ROAD P J EXTENSION
DAVANAGERE KARNATAKA,577002

TEST NAME	TECHNOLOGY	VALUE	UNITS
ANTI CCP (ACCP)	C.M.I.A	687.3	U/mL
Reference Range :-			
BRI	Interpretation		
Negative < 5	Absence of IgG autoantibodies to cyclic citrullinated peptides (CCP)		
Positive ≥ 5	Presence of IgG autoantibodies to cyclic citrullinated peptides (CCP)		

Clinical Significance :

1. Anti-Cyclic-Citrullinated-Peptide (Anti-CCP) titre is used for diagnosis and monitoring of Rheumatoid Arthritis (RA).
2. RA is one of the most common systemic autoimmune diseases characterised by chronic inflammation of the synovial joints and progressive joint degeneration eventually leading to disability of affected individuals.
3. The diagnosis of RA often relies on clinical manifestations and certain non-specific laboratory tests such as rheumatoid factor (RF) and C-reactive protein (CRP), which may be present in healthy elderly persons or in patients with other autoimmune and infectious diseases.
4. Whereas, Anti-Cyclic-Citrullinated-Peptide (Anti-CCP) Antibodies hold promise for early and more accurate detection of Rheumatoid Arthritis before the disease proceeds into irreversible damage.
5. Interference with pathologic levels of nonspecific IgG can not be excluded.
6. The anti-CCP test results can be false negative in patients with hypergammaglobulinemia.
Results from patients suffering from this disorder should not be used for diagnostic purposes.
7. Heterophile antibodies may interfere with the test results.
8. If results are inconsistent with clinical history additional testing is suggested to confirm the results.
9. Some specimens may not dilute linearly because of heterogeneity of autoantibodies with respect to physicochemical properties.
10. HAMA (Human Anti mouse antibodies) may also interfere with the results.
11. For diagnostic purposes, the results should always be assessed in conjunction with the patient's medical history, clinical examination and other findings.

References:

- Anti-CCP Reagent Kit Insert
- Feldmann M, Brennan FM, Maini RN. Rheumatoid arthritis.Cell 1996;85:307-3102.
- Landewé RB. The benefits of early treatment in rheumatoid arthritis:confounding by indication, and the issue of timing. Arthritis Rheum 2003;48(1):1-5.

Please correlate with clinical conditions.

Method:- Fully Automated ChemiLuminescent Microparticle Immunoassay (C.M.I.A)

~~ End of report ~~

Sample Collected on (SCT) :18 Jun 2025 20:49

Sample Received on (SRT) : 19 Jun 2025 11:01

Report Released on (RRT) : 19 Jun 2025 15:05

Sample Type :SERUM

Labcode :1906040217/HBDN1 Dr Syeda Sumaiya MD(Path)

Barcode : EC372340



CONDITIONS OF REPORTING

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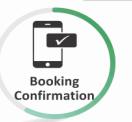
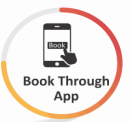
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 Thyroid	 Diabetes	 STDs	 Skin Care	 Hair Fall

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